

MANAGE THE PREGNANT/BREASTFEEDING PATIENT WITH AN UNSUPPRESSED VIRAL LOAD (VL ≥ 50)

Assess and manage possible causes of unsuppressed viral load (VL ≥ 50):

- Check adherence and dosing and give enhanced adherence support ↗ 173. Check if pregnant mother has been vomiting up medications. Encourage disclosure.
- Consider medication interactions: ask about other medications, especially TB and epilepsy treatment and common over-the-counter medications like: calcium, iron, antacids. If using any of these, manage possible medication interactions ↗ 118 or discuss with experienced ART doctor or HIV hotline ↗ 178.

Switch to the DTG-based regimen

- If *not* on TLD (or ALD), check if same day ART switch is appropriate ↗ 117.
- If on DTG-based regimen, continue below.

If breastfeeding, assess and manage baby:

- Do HIV test on baby same day.
- If baby not on prophylaxis currently, start/restart **AZT** 12 hourly for 6 weeks and **NVP** daily for *at least* 12 weeks ↗ 168 (only stop NVP once mother's VL < 50 or 4 weeks after final breastfeed).

Repeat mother's VL after 4 weeks (if VL done at delivery, VL can be repeated at the 6-week postnatal visit):

Second viral load result < 50

- Repeat VL as per routine VL monitoring:
- If pregnant: repeat VL at delivery.
 - If breast-feeding: repeat VL 6 monthly.

Second viral load result ≥ 50

- Increase efforts to resolve adherence¹ issues and address possible drug-drug interactions ↗ 173.
- Manage further according to duration of DTG-based ART:

On DTG for less than 2 years

On DTG for 2 years or more

- Assess adherence in last 6-12 months by checking script for pharmacy refills and notes for clinic appointment attendance¹.
- Have refills been collected > 80%² of time or has patient attended > 80%³ clinic visits?

No

Yes

Adherence considered poor.

Adherence considered good.
Has woman had 2 or more consecutive viral load results ≥ 1000 after starting DTG-based regimen?

No

Yes

Continue to support adherence and repeat VL in 3 months or at delivery if this is sooner:
if ≥ 2 viral loads ≥ 1000 after starting DTG-based regimen (with adherence > 80%):

Virological failure confirmed.

- Discuss with HIV expert, specialist, third line ART committee or HIV hotline ↗ 178.
- If VL ≥ 1000, monitor CD4 6 monthly.
- If CD4 ≤ 200, restart co-trimoxazole ↗ 113.

¹If available, also do drug level on urine or blood specimen: adherence is considered good if medications are detected in patient's urine/blood. ²Calculate adherence % for pharmacy refills: 'number of *actual refills done* during period assessed' ÷ 'number of months in period assessed'. Then x by 100. ³Calculate adherence % for clinic attendance: 'number of scheduled visits *actually attended* by patient during period assessed' ÷ 'number of scheduled visits during period assessed'. Then x by 100.